INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Tepotinib monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Regimen source
NCCP 00823
Version 2

Daily treatment with food

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Tepotinib blocks the MET growth signal in selected lung cancers with a MET exon 14 skipping change. In advanced cancer the aim is to control the cancer and its symptoms. The tumour test and previous treatment help determine whether it is suitable.

MET TARGETED THERAPY

Tepotinib

Blocks abnormal MET growth signalling

MET exon 14 change identified → MET signal blocked → Cancer growth slowed

Tepotinib inhibits the MET receptor. A MET exon 14 skipping change can keep this growth signal active; blocking it can slow or shrink selected lung cancers.

Report new cough or breathlessness, swelling, severe bowel symptoms, jaundice, palpitations or fainting promptly.

Your treatment aim

Which setting?

NCCP 00823a covers tepotinib on its own for adults with advanced non-small cell lung cancer carrying a confirmed MET exon 14 skipping change, after immunotherapy and/or platinum chemotherapy. It is not a first-line or adjuvant treatment guide.

NCCP 00823 v2 · official regimen · EMA Tepmetko product information, updated 08/10/2026

Dose and duration

The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects.

Evidence for this treatment

VISION found that tepotinib could shrink some advanced MET exon 14 skipping lung cancers, including cancers previously treated with other medicines. It was a single-arm study, so it does not show how much longer people live compared with another treatment. Study results describe groups and cannot predict your own outcome.

Single-arm evidence does not establish comparative survival benefit; first-line trial patients are outside this NCCP indication.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact the oncology team urgently for new or worsening cough, breathlessness or fever; marked swelling or sudden weight gain; severe diarrhoea, repeated vomiting or inability to drink; yellow skin or eyes, dark urine or severe abdominal pain; or palpitations or fainting. Severe breathing difficulty, collapse or swelling of the mouth or throat needs emergency help. Use your unit's temperature and 24-hour contact instructions.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Checks alongside continuous tablets

Relative stages, not equal time periods or booked appointments. NCCP does not specify a universal number of weeks between these checks; your team agrees the schedule.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00823 v2 · official regimen · EMA Tepmetko product information, updated 08/10/2026

THE WHOLE TREATMENT JOURNEY

Tepotinib monotherapy: treatment and checks

Your treatment pattern

Daily treatment with food

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Advanced MET exon 14 skipping non-small-cell lung cancer (NSCLC) · after prior systemic therapy

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before the first dose	Starting daily treatment	Ongoing reviews	Benefit or toxicity review	Change or stopping
Treatment NCCP 00823 · SMPC	Confirm prescription	Daily tepotinib with food	Continue authorised daily dose	Discuss benefit and side effects	Team authorises change or stopping
Cancer and eligibility NCCP 00823 · SMPC	MET test and previous treatment	Confirm prescription	Scans under the Consultant plan	Review response and symptoms	Agree the next plan
Blood and organ function NCCP 00823 · SMPC	Blood count, electrolytes, kidney and liver tests	Review results and early symptoms	Repeat regular tests	Extra tests for abnormalities	Follow unresolved effects
Heart NCCP 00823 · SMPC	electrocardiogram (ECG) when indicated	Check risk and interacting medicines	ECG/ electrolytes when indicated	Assess fainting or palpitations urgently	Confirm the follow-up plan

Care / check	Before the first dose	Starting daily treatment	Ongoing reviews	Benefit or toxicity review	Change or stopping
Symptoms and practical support NCCP 00823 · SMPC	Discuss lungs, swelling and bowel plan	Take with food; check supply	Review symptoms, weight and adherence	Authorised hold or dose decision	Continue contact advice and contraception

What each check is for

1 **Confirm eligibility**

Before the first dose

Treatment depends on a specific tumour alteration and previous treatment.

Ask / **act:** Document the NCCP 00823a indication and agreed prescription.

2 **Keep tests and symptoms connected**

At scheduled reviews and between visits

Blood tests can change before symptoms and symptoms can occur between appointments.

Ask / **act:** Confirm who orders tests and reviews results; give an urgent contact plan.

3 **Possible interstitial lung disease (ILD) or liver injury**

At any time

Suspected lung inflammation and significant liver abnormalities can require an immediate hold.

Ask / **act:** Urgent clinical assessment and an authorised treatment decision.

4 **Benefit or toxicity changes**

At treatment reviews

There is no fixed chemotherapy course length.

Ask / **act:** Agree continuation, an authorised hold/restart, or the next treatment plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Before and during treatment

Tests and symptoms guide treatment decisions.

Before starting

Confirm the MET result, previous treatment, dose, other medicines and emergency contact. Check blood count, electrolytes, kidney and liver function; ECG if clinically indicated. Discuss pregnancy, contraception and breastfeeding, and perform a pregnancy test when applicable.

Ongoing reviews

Repeat blood count, electrolytes, kidney and liver tests at the team's agreed reviews. ECG is used when clinically indicated. Report new lung symptoms, swelling, diarrhoea or vomiting between visits. A rise in creatinine needs assessment because the medicine can change the test result without necessarily damaging the kidneys. Scans follow the Consultant's plan.

Taking tepotinib

Follow your own prescription.

Food and missed doses

Take at the same time each day with food and swallow whole. If a dose is missed, take it when remembered on the same day unless the next dose is due within 8 hours; then skip it. Do not double a dose. Ask your pharmacist if the timing is uncertain or if you vomit; do not take a replacement without advice. If swallowing is difficult, ask about the approved water-dispersion method.



Medicine interactions

Ask the pharmacist to check all prescribed medicines, non-prescription medicines and supplements before changes. Tepotinib can affect medicines including dabigatran, digoxin, rosuvastatin and metformin. Supportive medicines may also affect heart rhythm; individual advice is needed.

EMA Tepmetko product information, updated 08/10/2026

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Daily tablets, tests and treatment decisions

No dates or personal information are needed. Confirm your actual appointments with the team.

Treatment or check	Timing in this view
Preparation	**Before the first dose** Confirm the MET result, prior treatment, prescription, blood tests and medicines.
Daily oral treatment	**Starting daily treatment → Ongoing reviews → Benefit or toxicity review** The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects.
Tests and symptoms	**Starting daily treatment → Ongoing reviews → Benefit or toxicity review → Change or stopping** Regular blood tests, symptom review and electrocardiogram (ECG) when indicated; report changes between visits.
Next treatment decision	**Change or stopping** Continue only while benefit is observed and the team considers the treatment appropriate.

Take at the same time each day with food and swallow whole. If a dose is missed, take it when remembered on the same day unless the next dose is due within 8 hours; then skip it. Do not double a dose. Ask your pharmacist if the timing is uncertain or if you vomit; do not take a replacement without advice. If swallowing is difficult, ask about the approved water-dispersion method.

Columns show sequence, not equal periods. A hold does not authorise you to restart from this timeline.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Support during treatment

Report changes early so your team can help.

Swelling and weight changes

Report swelling early and discuss changes in weight, movement or breathing. The team will assess the cause and agree treatment; do not start a water tablet or change tepotinib yourself.

EMA Tepmetko product information, updated 08/10/2026

Diarrhoea and fluids

Agree a diarrhoea treatment plan before starting. Begin the prescribed anti-diarrhoeal medicine at the first loose stool as instructed, and maintain fluids if able. Persistent diarrhoea, weakness or reduced urine needs prompt advice.

Nausea medicines

Tepotinib has minimal-to-low oral emetic risk. Agree an as-needed anti-sickness plan with the team, including medicine-interaction and heart-rhythm checks; no routine infusion premedication is specified.

NCCP 00823 v2 · official regimen · NCCP systemic anticancer therapy (SACT) Antiemetic Guidance V6 (2025), oral therapy Table 5

Medicine review

EMA Tepmetko product information, updated 08/10/2026

Pregnancy, fertility and breastfeeding

Avoid pregnancy during treatment. Effective contraception is needed during treatment and for at least one week after the last dose; women using hormonal contraception should also use a barrier method. Men with a partner who could become pregnant should use barrier contraception for the same period. Do not breastfeed during treatment or for at least one week afterwards. Human fertility effects are not known; discuss fertility before starting.

When to get urgent help

Do not wait for the next appointment.

Call urgently

Contact the oncology team urgently for new or worsening cough, breathlessness or fever; marked swelling or sudden weight gain; severe diarrhoea, repeated vomiting or inability to drink; yellow skin or eyes, dark urine or severe abdominal pain; or palpitations or fainting. Severe breathing difficulty, collapse or swelling of the mouth or throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions.

When you call

Give the medicine name, last dose and symptoms. Do not delay calling while gathering information.

What studies tell us

Your own decision

Ask how closely the trial population matches your situation and discuss alternatives and remaining uncertainties.

VISION long-term follow-up, Mazieres et al., JAMA Oncology 2023 · EMA Tepmetko product information, updated 08/10/2026

When treatment changes

The team authorises continuation, a pause, restart or stopping.

Review benefit and side effects

Agree who will review test results and confirm the dose. A source reduction to 225 mg is a clinician decision, not an instruction to halve tablets yourself.

Follow-up

Confirm the next treatment or follow-up plan, symptom advice and precautions after the final dose.

Practical support

Swelling and weight changes

Diarrhoea and fluids

Nausea medicines

Medicine review

Pregnancy, fertility and breastfeeding

Taking tablets / attending treatment

Practical treatment details

Supplies and reviews

Arrange repeat prescriptions before tablets run out.

The tablet regimen itself needs no infusion line or pump.

Questions to ask

Is this a fixed course of chemotherapy?

No. These targeted tablets are taken daily without a planned rest week, while they are helping and your team considers treatment appropriate.

Why does a trial mention 500 mg?

The VISION trial's 500 mg salt dose corresponds to 450 mg active tepotinib. The prescribed tablets contain 225 mg active tepotinib each; follow your own prescription.

What if I forget or vomit a dose?

Take a forgotten dose when remembered on the same day unless the next dose is due within 8 hours; then skip it. Never double up. Ask your pharmacist about vomiting or uncertain timing before taking any replacement.

Why might my creatinine test change?

Tepotinib can change how creatinine is handled by the kidneys, but real kidney problems can also occur. The team must assess the change; neither explanation should be assumed.

Can I change the dose when I feel better?

Only your team can authorise restarting or changing the dose after side effects. Feeling better does not establish that blood tests or other checks have recovered.

Explain the plan back: Can you explain why the MET result and previous treatment matter, how you take the tablets with food, what to do if a dose is missed, and which symptoms mean you should call today?

Advanced MET exon 14 skipping lung cancer NCCP 00823a covers tepotinib on its own for adults with advanced non-small cell lung cancer carrying a confirmed MET exon 14 skipping change, after immunotherapy and/or platinum chemotherapy. It is not a first-line or adjuvant treatment guide.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Tepotinib alone · daily treatment

Active medicines: tepotinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Report swelling early and discuss changes in weight, movement or breathing. The team will assess the cause and agree treatment; do not start a water tablet or change tepotinib yourself.
- Agree a diarrhoea treatment plan before starting. Begin the prescribed anti-diarrhoeal medicine at the first loose stool as instructed, and maintain fluids if able. Persistent diarrhoea, weakness or reduced urine needs prompt advice.
- Tepotinib has minimal-to-low oral emetic risk. Agree an as-needed anti-sickness plan with the team, including medicine-interaction and heart-rhythm checks; no routine infusion premedication is specified.
- Ask the pharmacist to check all prescribed medicines, non-prescription medicines and supplements before changes. Tepotinib can affect medicines including dabigatran, digoxin, rosuvastatin and metformin. Supportive medicines may also affect heart rhythm; individual advice is needed.

- Avoid pregnancy during treatment. Effective contraception is needed during treatment and for at least one week after the last dose; women using hormonal contraception should also use a barrier method. Men with a partner who could become pregnant should use barrier contraception for the same period. Do not breastfeed during treatment or for at least one week afterwards. Human fertility effects are not known; discuss fertility before starting.
- The written diarrhoea prescription and escalation instructions should be confirmed before treatment. NCCP provides loperamide dosing or local-policy options; these must be individualised.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Advanced MET exon 14 skipping lung cancer National Cancer Control Programme (NCCP) 00823a covers tepotinib on its own for adults with advanced non-small cell lung cancer carrying a confirmed MET exon 14 skipping change, after immunotherapy and/or platinum chemotherapy. It is not a first-line or adjuvant treatment guide.

NCCP 00823 · VERSION 2

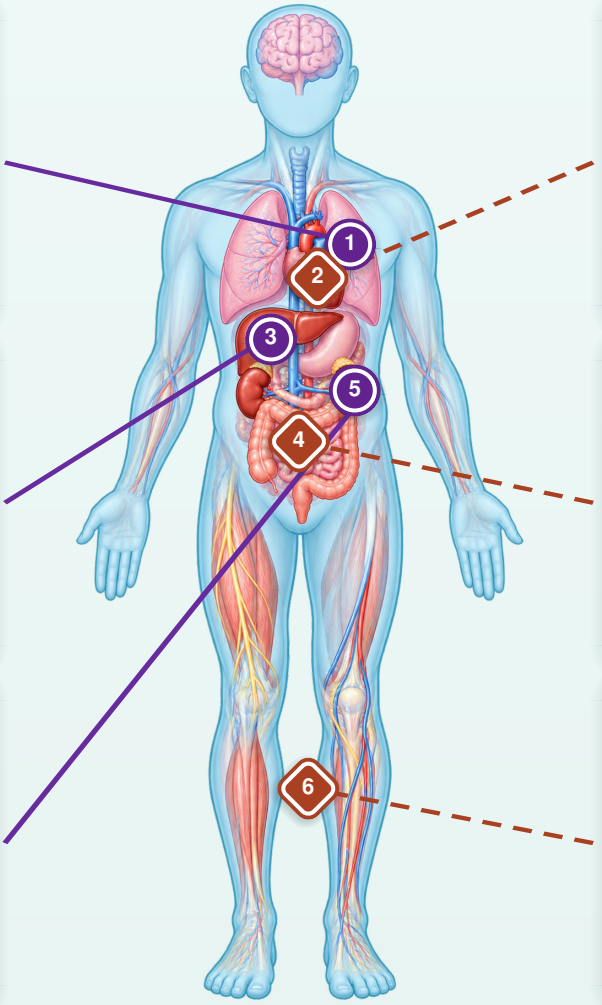
Tepotinib monotherapy

The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 New cough or breathlessness

tepotinib

Possible lung inflammation needs urgent assessment.

2 Palpitations or fainting

tepotinib

Possible heart-rhythm changes need assessment.

3 Yellow skin or eyes

tepotinib

Liver changes may be found in tests or cause symptoms.

4 Diarrhoea or vomiting

tepotinib

Early support helps prevent dehydration.

5 Kidney-test changes

tepotinib

Creatinine changes need assessment, including causes other than kidney injury.

6 Swelling and weight gain

tepotinib

Fluid retention can affect movement or breathing; report changes.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team urgently for new or worsening cough, breathlessness or fever; marked swelling or sudden weight gain; severe diarrhoea, repeated vomiting or inability to drink; yellow skin or eyes, dark urine or severe abdominal pain; or palpitations or fainting. Severe breathing difficulty, collapse or swelling of the mouth or throat needs emergency help. Use your unit's temperature and 24-hour contact instructions. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00823 v2 · official regimen**
   https://healthservice.hse.ie/documents/6868/823_V2_Tepotinib_Therapy.pdf
2. **EMA Tepmetko product information, updated 08/10/2026**
   https://www.ema.europa.eu/en/documents/product-information/tepmetko-epar-product-information_en.pdf
3. **VISION long-term follow-up, Mazieres et al., JAMA Oncology 2023**
   https://jamanetwork.com/journals/jamaoncology/fullarticle/2805800
4. **VISION treatment sequencing: ≥3-year follow-up, ESMO 2025 abstract 1995P**
   https://www.annalsofoncology.org/article/S0923-7534(25)03539-2/fulltext
5. **VISION sustained-response subset, ASCO 2026 abstract 8612**
   https://ascopubs.org/doi/10.1200/JCO.2026.44.16_suppl.8612
6. **NCCP V6 Table 5: as-needed antiemetics for minimal-low oral therapy; NCIS V8 parenteral schedules do not impose a routine oral prescription**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
7. **NCCP NCIS supportive antiemetics V8: parenteral guidance**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf